INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Encorafenib and binimetinib for advanced BRAF-mutated melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00563
Version 4

Reference calendar for daily medicine and reviews

Treatment stages

01 **Continuous oral treatment**
Daily medicines

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	6
03	Symptoms and supportive care	11
04	Sources and further information	18

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Encorafenib and binimetinib for advanced BRAF-mutated melanoma · National Cancer Control Programme (NCCP) 00563

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

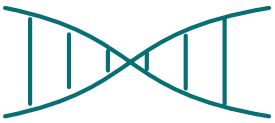
Medicines in this regimen

Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

ORAL BRAF INHIBITOR

Encorafenib

Braftovi



Blocks altered BRAF signalling

Encorafenib blocks altered BRAF signalling in eligible melanoma. Binimetinib acts at a different step in the same growth pathway. Each medicine retains its own dosing and safety instructions.

BRAF V600 confirmed → BRAF signal blocked → Partner MEK medicine

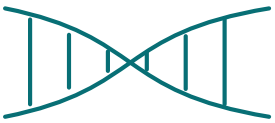
What to watch for

A change to binimetinib can change the encorafenib plan. Have interactions checked and avoid grapefruit.

ORAL MEK INHIBITOR

Binimetinib

Mektovi



Blocks downstream MEK signalling

Binimetinib inhibits MEK proteins in the growth-signalling pathway. Alongside effects on cancer cells, it can affect muscle, heart and retinal tissue. The prescribed blood and heart checks and prompt review of new eye symptoms help detect these effects.

MEK inhibited → Cancer-growth signal reduced → Muscle / heart / eye checks

What to watch for

Report vision change or muscle pain/weakness promptly; keep creatine kinase (CK), kidney and heart checks even when you feel well.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

What the diagnosis says

Record the original melanoma site and pathology findings. Breslow thickness and ulceration describe features of the skin tumour; stage describes its extent. Melanoma arising in the eye or a mucosal surface may need a different pathway.

BRAF and other tumour tests

A BRAF V600 result identifies whether the targeted medicines in this plan can act on the tumour. Keep the laboratory result and date. A tumour change is not, on its own, evidence of an inherited condition.

Surgery, radiotherapy and medicine have different roles

Ask whether medicine is intended to reduce recurrence after complete surgery or control cancer that remains. Add any operation or radiotherapy to the plan with its own purpose, timing and risks.

Stage and response are different

Stage describes the extent of disease. Response describes a change in measurable cancer during treatment. After complete removal there may be no visible tumour to measure; follow-up then looks for recurrence and treatment effects.

A new skin change still needs review

Report a new or changing mole, a sore that persists, a new lump or an unexplained symptom. A photo or a good response in one area cannot rule out a separate problem.

Your medicines and the overall plan

The schedule in this setting

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

How the treatment works

Encorafenib blocks altered BRAF signalling in eligible melanoma. Binimetinib acts at a different step in the same growth pathway. Each medicine retains its own dosing and safety instructions. Binimetinib inhibits MEK proteins in the growth-signalling pathway. Alongside effects on cancer cells, it can affect muscle, heart and retinal tissue. The prescribed blood and heart checks and prompt review of new eye symptoms help detect these effects.

The role of earlier or later treatment

Ask how the medicine fits with previous or planned local treatment. The timing of an operation, radiotherapy and a tablet course should be written in one agreed plan.

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what outcome this treatment aims to improve for your diagnosis and what alternatives exist. Results from a study group help a discussion but cannot promise your own result.

Alternatives and review points

Agree the next decision point and what would make the plan change. Ask about other suitable treatments, trials and symptom care. Support for comfort and function can continue alongside cancer treatment.

Help with daily life

Tell the team what is hardest in daily life: travel, work, caring duties, costs, sleep or confidence. Ask who can help and how to contact them.

Shared understanding

Before going home, check that you can describe your medicine schedule, next tests and urgent-contact route. Ask for an interpreter, larger print or a supporter if that would help.

What the evidence means for your treatment

Discuss the evidence for your diagnosis, tumour test results and treatment setting.

Ask which outcomes matter in your setting and what benefit is realistic for you. No survival percentage from another regimen, dose combination or disease setting is assigned here.

SACTCHECK · CHAPTER 02

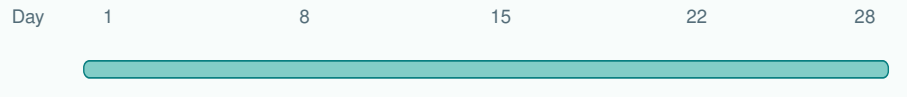
Your schedule, tests and appointments

Encorafenib and binimetinib for advanced BRAF-mutated melanoma · NCCP 00563

The overall treatment plan

Encorafenib and binimetinib for advanced BRAF-mutated melanoma

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAILY MEDICINES

Continuous oral treatment

No preset cycle total

Encorafenib daily + binimetinib twice daily, continuously; reviewed regularly.

Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00563 v4: current regimen source

THE WHOLE TREATMENT JOURNEY

Encorafenib and binimetinib for advanced BRAF-mutated melanoma: treatment and checks

NCCP 00563: adults with advanced unresectable or metastatic melanoma with a BRAF V600 mutation.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00563	Confirm BRAF V600 and treatment setting. Full blood count, kidney/liver tests, creatine kinase (CK) and creatinine, blood pressure, electrocardiogram (ECG), a heart-pumping assessment by echo or multigated acquisition (heart pumping) scan (MUGA), and skin examination.	Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest computed tomography (CT) every six months. Response scans follow the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen. Agree continuing symptom/organ checks, the next melanoma assessment and medicine-specific pregnancy precautions.
Cancer, skin and daily function NCCP 00563	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Medicines and continuing precautions NCCP 00563	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.

What each check is for

1 Continuous oral treatment

Daily medicines

Encorafenib and binimetinib block two points in the BRAF-MEK growth pathway to help control eligible advanced melanoma. The team reviews response, symptoms and side effects to decide whether the treatment continues.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

The usual method is to swallow the prescribed capsules/tablets whole with water, with or without food. If swallowing is difficult, ask the pharmacist for the source-permitted preparation: Braftovi capsules may be opened into about 20 mL of apple puree for immediate use, and Mektovi 15 mg tablets may be dispersed in about 10 mL of water, orange juice or apple juice, taken immediately, followed by a further 10 mL rinse. Do not assume this applies to other tablet strengths or improvise a preparation. Keep doses at consistent times. Avoid grapefruit and grapefruit juice, and have all medicines and supplements checked.

STEP 2

Missed, vomited or held treatment

For a missed encorafenib dose, take it only if more than 12 hours remain before the next dose. For binimetinib, do not take a missed dose if fewer than six hours remain before the next; ask the pharmacist if timing is uncertain. Do not double doses. If either medicine is vomited, do not take an extra dose; take the next scheduled dose unless the team advises otherwise.

STEP 3

Appointments and supply

Plan blood tests, travel and medicine collection in advance. If you need an interpreter or someone to help at home, tell the team before the appointment.

STEP 4

Paper copies and contact information

Ask for your latest schedule and contact details on paper. Replace old versions when treatment changes so everyone helping you uses the same plan.

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Practical preparation

Bring your medicine list, allergy information and questions. Tell the team about practical problems with travel, swallowing, wound care or remembering doses before treatment starts.

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Your stage, symptoms and treatment determine follow-up. Write down the skin and lymph-node checks, any body or brain scans and the service arranging them. Confirm your own dates with the team.



Continuing medicine precautions

Continue skin surveillance for six months after stopping as specified by the regimen. Agree continuing symptom/organ checks, the next melanoma assessment and medicine-specific pregnancy precautions.

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

The complete paper version

Use this handbook as your paper record. Keep the current schedule, medicines, tests and contacts together, and replace outdated pages after a change. The matching QR link is optional.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Encorafenib and binimetinib for advanced BRAF-mutated melanoma · NCCP 00563

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Daily oral treatment	 **Days 1–28** Encorafenib daily + binimetinib twice daily, continuously; reviewed regularly.
Safety reviews as scheduled	 **Days 1–28** Keep the specific blood, organ and skin-review appointments; they do not all follow the same interval.
Symptoms and support	 **Days 1–28** Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.

Timing note 1: Take encorafenib once daily and binimetinib twice daily, around 12 hours apart, continuously unless the team tells you to hold them. Treatment continues while beneficial and tolerable. A 28-day review calendar does not mean a routine break or a fixed total number of cycles.

Timing note 2: For a missed encorafenib dose, take it only if more than 12 hours remain before the next dose. For binimetinib, do not take a missed dose if fewer than six hours remain before the next; ask the pharmacist if timing is uncertain. Do not double doses. If either medicine is vomited, do not take an extra dose; take the next scheduled dose unless the team advises otherwise.

Timing note 3: Temporary holds may need coordinated changes to both medicines; no patient-led dose adjustment.

Timing note 4: The regimen is advanced melanoma; it is not an adjuvant treatment template.

Timing note 5: No trial outcome percentage is assigned without confirming the exact population, comparison and follow-up.

Timing note 6: Tumour lysis syndrome is a source-listed risk, especially with high tumour burden, kidney problems or dehydration. The team decides whether extra hydration and blood checks are needed.

Why these bands stay broad: This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00563 v4: current regimen source ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1–28: Every day in the reference calendar

Encorafenib daily + binimetinib twice daily, continuously; reviewed regularly. Use the written medicine plan; report new symptoms and keep tests even during a treatment hold.

Support around the medicines

1 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

2 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

3 Pregnancy and fertility precautions

Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.

4 Eating, activity and practical help

Let the team know if cramps, altered taste or poor appetite make eating or moving difficult. Ask for practical food and activity advice that fits your symptoms and any other illness.

5 Hydration and tumour lysis assessment

The team checks whether tumour burden, kidney problems or dehydration call for additional precautions and blood tests. Keep the individual fluid plan; do not force fluids if you have a restriction. Report reduced urine, persistent vomiting or marked weakness promptly.

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Confirm BRAF V600 and treatment setting. Full blood count, kidney/liver tests, creatine kinase (CK) and creatinine, blood pressure, ECG, a heart-pumping assessment by echo or MUGA, and skin examination.

During treatment

Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest CT every six months. Response scans follow the individual plan.

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Getting the result

Keep a named contact for bookings and results. If an expected appointment or result is missing, call that service; do not take silence as confirmation that everything is complete.

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Eating, drinking and activity

Report nausea, mouth discomfort, diarrhoea or weight loss before they make eating and drinking difficult. The team can adjust symptom support and arrange dietitian or rehabilitation advice.

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Protect exposed skin with clothing, a hat and shade. Use broad-spectrum sunscreen with sun protection factor (SPF) 30 or higher and reapply as directed. Avoid sunbeds. Cloudy weather does not remove ultraviolet exposure.

Know the pattern of your skin

Ask the team to show you how to check your skin and treated area. Use a mirror or a helper for areas you cannot see. Report a new lump, a changing lesion or a sore that does not heal.

Photographs are optional

A private, dated photograph can help describe a change if your team finds it useful. Keep the lighting and size reference consistent. Photographs support a discussion; they do not replace examination and need not be uploaded.

Wounds, nails and sensitive skin

Follow the wound and skin-care instructions for the treated area. Ask before adding a cream or remedy. Increasing redness, discharge, pain, bleeding or difficulty using the area needs assessment.

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.

Medicine-specific precautions

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Questions worth asking

When are my next tests?

Blood count, kidney/liver tests monthly. CK and creatinine monthly for the first six months, then as indicated. ECG at four weeks, then every 12 weeks and after dose changes; heart-pumping assessment every three months. Skin assessment every 60 days, head/neck examination every three months and chest CT every six months. Response scans follow the individual plan. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

Your diagnosis and treatment setting NCCP 00563: adults with advanced unresectable or metastatic melanoma with a BRAF V600 mutation.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Encorafenib + Binimetinib

Emetogenic risk: Moderate to high (oral)

Medicine	Dose / route	When
Individual anti-sickness / rescue plan	To be completed by the prescribing team if needed	As specified in the individual plan; no routine schedule issued here Source emetogenic category and local policy guide the decision. No medicine, dose or administration order is selected by this draft.

Additional phase-specific support

- Assess tumour lysis risk when tumour burden, kidney problems or dehydration raise concern; fluids and additional tests are individual prescriptions. Both oral components are moderate-to-high emetic risk in the source.
- Use effective contraception during treatment and for at least one month after the last dose. Hormonal contraception needs an additional or alternative barrier method because interactions can reduce effectiveness. Discuss fertility beforehand and follow the team's breastfeeding instructions. Report a possible pregnancy promptly.
- Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.
- This is not an issued prescription. The clinician and pharmacist must complete and authorise any supportive medicines.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy / fever

Both medicines

Fatigue and headache can occur. Fever, marked weakness, dizziness or new neurological symptoms needs advice; do not assume it is a harmless drug effect.

3 Heart / circulation

Both medicines

Keep ECG and heart-pumping checks. Report palpitations, fainting, swollen ankles or new breathlessness; severe chest pain is an emergency.

5 Liver

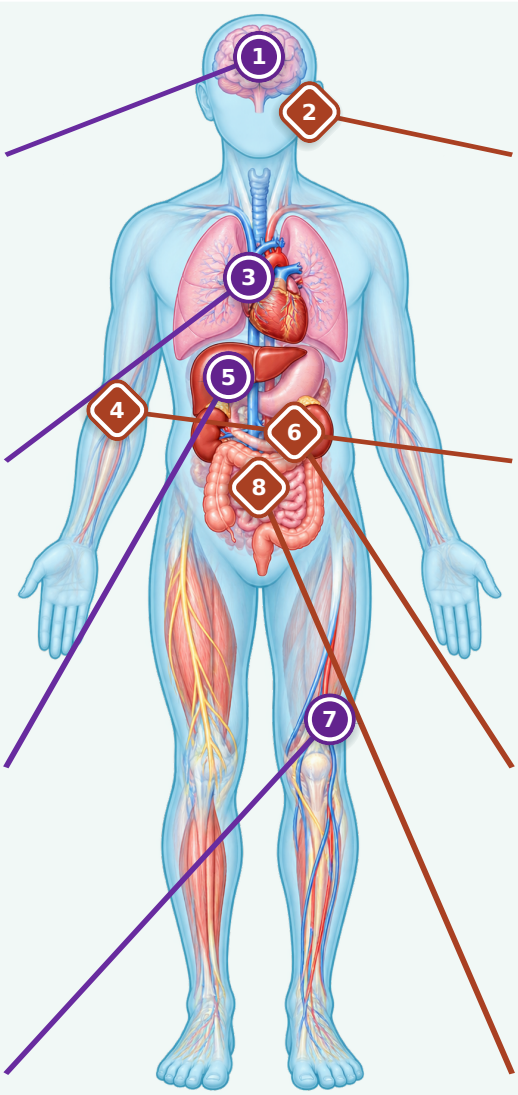
Both medicines

Liver changes may have no early symptoms, so attend monthly tests. Report yellow skin or eyes, dark urine or upper-abdominal pain.

7 Muscles / strength

Binimetinib

CK can rise before or with muscle injury. Report muscle pain, weakness or dark urine, and keep the planned blood tests.



2 Eyes / vision

Both medicines

Blurred vision, spots, altered colour vision or eye pain needs prompt assessment. Retinal problems may occur with binimetinib.

4 Skin / new lesions

Both medicines

Report new growths or changes in existing spots and attend skin checks. Severe rash, blistering or peeling needs urgent help.

6 Kidneys / fluids

Binimetinib / combination

Kidney tests accompany CK monitoring. Reduced urine, dehydration or dark urine with muscle symptoms needs urgent advice.

8 Stomach / bowels

Both medicines

Nausea and diarrhoea may occur. Persistent vomiting, inability to drink, severe abdominal pain, black stools or bleeding needs assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the team promptly for new visual symptoms, chest pain, breathlessness, palpitations, faintness, severe muscle pain or weakness, dark urine, bleeding, severe rash, persistent diarrhoea or severe abdominal pain. Do not continue one medicine at an unchanged dose if the other has been held without a specific team instruction. Report markedly reduced urine, persistent vomiting, new muscle cramps or weakness, or palpitations promptly; the team may need to check salts, kidney function and tumour lysis risk.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

SACTCHECK · CHAPTER 04

Sources and further information

Encorafenib and binimetinib for advanced BRAF-mutated melanoma · National Cancer Control Programme (NCCP) 00563

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00563 v4: current regimen source**
   https://healthservice.hse.ie/documents/6744/563_Encorafenib-and-Binimetinib-Therapy.pdf
2. **EMA: Braftovi medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/braftovi
3. **EMA: Mektovi medicine overview**
   https://www.ema.europa.eu/en/medicines/human/EPAR/mektovi
4. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
5. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
6. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
7. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet
8. **NCI: skin cancer — patient information**
   https://www.cancer.gov/types/skin/patient/skin-treatment-pdq
9. **HSE SunSmart: protection from sun and sunbeds**
   https://www.hse.ie/conditions/cancer/protect-skin-sun-sunbeds/